

Twin Pregnancy 08. Intrapartum Execution and Emergencies

Companion reading handout for simultaneous listening.

Audio course on twin pregnancy. Episode eight. Intrapartum execution and emergency thinking.

Source base

This episode is based on the local twin delivery protocol, the Israeli delivery position paper, the intrapartum fetal monitoring guideline, the Israeli fetal heart rate monitoring position paper, and the local Pitocin, vacuum, anti-D, and postpartum hemorrhage protocols.

Opening frame

Let us begin with the deepest idea.

A planned vaginal twin delivery is not an ordinary vaginal birth with an extra baby. It is a controlled high-risk procedure with a predictable moment of instability: the interval after twin A delivers and before twin B is born.

Preparation is therefore not bureaucracy. It is how the team buys time before the unstable moment arrives.

Start with location

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Twin delivery should occur where rapid operative delivery is possible. The local protocol places vertex-vertex deliveries in rooms adjacent to the operating rooms, with senior physician discretion about whether to deliver in the operating room. If twin B is not vertex, the mother is transferred to the operating room before delivery.

Why does location matter? Because the second twin can convert from reassuring to urgent quickly. Cord prolapse, abruption, malpresentation, and fetal heart rate deterioration are time-sensitive. The correct room is part of the intervention.

Now staffing

Now staffing.

The local protocol requires notification of midwives, anesthesia, and neonatology, and mandates senior obstetric presence. Two neonatal resuscitation stations should be ready. This reflects the fact that each neonate may need independent assessment and resuscitation, and the second baby may be more compromised.

Anesthesia should be available, with a strong advantage to epidural anesthesia. Epidural does more than treat pain. It allows cooperation, internal maneuvers, uterine relaxation strategies, and rapid extension for cesarean if needed.

Now monitoring

Now monitoring.

During active labor, both fetuses require continuous monitoring. The core problem is identity. External monitors can lose signal, switch signal, or record the same fetus twice. The intrapartum fetal monitoring guideline emphasizes clearly assigning each trace to each twin and documenting it. It suggests offsetting twin 2 by 20 beats per minute so the traces are easier to distinguish.

The local protocol adds practical safeguards. A physician may use ultrasound to locate heart rates. When possible, amniotomy for twin A and placement of a scalp electrode can improve trace reliability. Maternal pulse should be checked intermittently or continuously to avoid confusing maternal heart rate with fetal heart rate.

Do not rely on ultrasound as a substitute for continuous fetal monitoring. Ultrasound can show cardiac activity, but it does not give variability, recurrent decelerations, sinusoidal patterns, or the evolution of hypoxia.

Now induction and augmentation

Now induction and augmentation.

Induction can be performed in twins after senior consideration, using accepted methods. Pitocin requires respect. The local twin protocol recommends lower initial dosing in some circumstances, and the Pitocin protocol requires senior review after a liter of oxytocin solution. The reason is not that oxytocin is uniquely toxic in twins. It is that uterine overdistension, monitoring difficulty, and rapid need for intervention make tachysystole and fetal compromise more consequential.

Now the birth of twin A

Now the birth of twin A.

After twin A delivers, the team should not relax. This is when twin B becomes the focus.

Immediately determine twin B presentation and position, usually with ultrasound, and confirm heart rate. If twin B is cephalic and the tracing is reassuring, the team may await descent and delivery, using oxytocin when appropriate.

If twin B is non-cephalic, experienced operators often proceed with breech extraction rather than external cephalic version. If uterine relaxation is needed for manipulation, nitroglycerin or terbutaline should be available according to the local protocol.

There is no absolute evidence-based time limit between delivery of twin A and twin B, but the local protocol recommends not exceeding about 30 minutes. The reasoning is practical. The longer the interval, the more opportunity for presentation change, cord prolapse, abruption, and fetal compromise.

Now blood and hemorrhage preparation

Now blood and hemorrhage preparation.

Twin pregnancy stretches the uterus. A stretched uterus may not contract well after delivery. Therefore, blood should be available, intravenous access should be secure, and uterotonics should be ready. The PPH position paper lists multiple gestation as a postpartum hemorrhage risk factor through uterine overdistension.

Placenta should be sent to pathology. In twins, placental examination can confirm chorionicity, reveal vascular anastomoses, clarify cord insertions, and explain growth discordance, TTTS, TAPS, or fetal compromise.

Now anti-D

Now anti-D.

The local anti-D protocol lists higher dosing for twin pregnancy and after the birth of twins or more. This is a small practical detail, but it reflects a large principle. Twin delivery can involve more fetomaternal hemorrhage exposure, and postpartum immunoprophylaxis should match the risk context.

Now emergencies

Now emergencies.

If twin B develops prolonged bradycardia after twin A delivers, the team must rapidly decide whether vaginal extraction is faster and safer than cesarean. This decision depends on presentation, station, membranes, operator skill, anesthesia, and whether the operating room is already active. The reason twin delivery requires senior presence is that this decision cannot be made from an algorithm alone.

If cord prolapse occurs, relieve pressure, call for immediate delivery, and choose the fastest safe route. If twin B is breech and accessible with full dilation and skilled hands, breech extraction may be the fastest safe route. If not, cesarean may be required.

If abruption occurs after twin A, fetal status and maternal bleeding guide urgency. Again, preparation matters because this is a known second-twin risk.

If combined delivery occurs, meaning vaginal delivery of twin A and cesarean delivery of twin B, it is not automatically evidence of poor planning. It is a recognized complication of twin delivery. The goal is to reduce its probability by selection and preparation, and to respond quickly if it occurs.

Pause and summarize.

Intrapartum twin care is built around identity, access, and speed. Identify each fetus on monitoring. Keep ultrasound, anesthesia, blood, neonatal teams, uterotonics, and an operating room available. After twin A, immediately reassess twin B. The second twin is the reason the whole delivery was prepared differently.

Now final synthesis for the entire course

Now final synthesis for the entire course.

Key point

First, twin pregnancy begins with classification. DCDA, MCDA, and MCMA are not labels. They are risk maps.

Second, monochorionic disease is vascular disease of a shared placenta.

Third, preterm birth in twins is common because multiple pathways converge: stretch, cervix, membranes, fetal disease, and indicated delivery.

Fourth, delivery timing follows chorionicity, and delivery route follows twin A, twin B, weight, gestational age, monitoring, and operator skill.

Finally, safe twin care is anticipation. The best teams look calm in emergencies because they prepared for the predictable moments of instability before they happened.

End of episode eight. End of the twin pregnancy audio course.